



WNHS PROCEDURE	
MBU Discharge Planning Procedure	
Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mother Baby Unit (MBU)
This document should be read in conjunction with the Disclaimer .	

1. Aim

The purpose of this guideline is to ensure all patients have a discharge plan prior to being discharged from the Mother and Baby Unit (MBU). The treating team will ensure that the patient, their Personal Support Person (PSP), the receiving community service and General Practitioner (GP) has an understanding of the Discharge Plan. The Community Mental Health Team will receive a copy of the Care Transfer Summary and the final discharge summary, consistent with the [North Metropolitan Health Service \(NMHS\) Discharge Summary Policy](#), the [WNHS Discharge of Patient Policy](#), the [Chief Psychiatrist's Guidelines 2015 e- \(e\)](#). The preparation, review and revision of treatment, support and discharge plans and the [Chief Psychiatrist's Clinical Care Standards- Transfer of Care](#). The nominated GP will receive a copy of the discharge summary.

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☐
Breach National/State/Mandatory Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Key Points

The Treatment, Support and Discharge Plan (TSDP) is developed in collaboration with the patient, the patient's partner/PSP and other relevant stakeholders to facilitate recovery. It is part of the continuum of care that commences at the time of admission. The Treatment, Support and Discharge Plan (TSDP) is holistic and considers, at a minimum, the following key points:

- patient's own goals of admission
- mental health status including relapse triggers and markers
- risk assessment
- psychotropic medications
- ongoing therapy recommendations
- the patient's physical health status (including contraception/post birth obstetric follow-up)
- any co-morbid drug/alcohol issues
- the patient's living situation and social supports
- the wellbeing of the infant, partner, siblings and other family members or carers (this may include consideration of parenting support, childcare needs and attention to specific physical, developmental and mental health needs of the infant).

4. Process

The following process will be adopted for discharge planning.

On Admission

- The discharge planning process commences on admission and planning is progressed throughout the hospital stay to ensure ongoing continuity of care.
- An estimated date of discharge is given on admission following assessment of the patient by the treating team.

Throughout Hospital Stay

- The patient's healthcare record must reflect a coordinated approach by the various clinicians in recording and integrating the assessment of the patient's readiness for discharge.
- Options for community based follow up will be identified early in the hospital stay, discussed with the patient and their PSP, documented in the patient's healthcare record and a referral sent as appropriate.
- The PSP, relevant community mental health representatives and other stakeholders as indicated will be invited to be involved in preparing and planning for discharge.

Prior to Discharge

- If there are complex care needs on discharge, the medical staff will contact the patient's named GP by telephone to ensure they are aware of treatment and discharge plan.
- If there is no named GP, staff will assist the patient to identify a GP and make contact.
- The community service responsible for coordinating the patient's care after discharge must be informed of the date of the patient's impending discharge and essential documentation forwarded promptly.

- Liaison with the MBU Clinical Nurse Consultant will occur post discharge. A joint meeting with the community case manager to facilitate handover of care may be indicated particularly for patients with complex needs.
- All patients will receive a minimum of a telephone call within 7 days of discharge. If they are not contactable NOK/PSP/community services will be contacted. Any risk issues will be discussed with the Psychiatrist and escalated appropriately either through MHERL and/or their GP.
- If required, social work assessment is made available for identification of community supports for the patient in the first instance and carers.
- In the 72 hours prior to discharge (unless discharge is planned suddenly), all patients will be given a clinical psychology discharge package of self-report measures to complete. If discharged on a weekend, nursing staff to review the self-report measures and notify on-call registrar if concerned. The results will be entered in the healthcare record within 48 hours prior to discharge.

At Discharge

At discharge, the patient and their PSP (where appropriate) will be provided with the following written information:

- Medication requirements
- Medical referrals and arrangements for continuing psychiatric care (including appointment times, contact details). A follow-up appointment with community mental health services (or an appropriate alternative service) must be scheduled within seven days of discharge.
- The Treatment, Support and Discharge Plan (TSDP) and the NACs will be given to the patient on discharge. If not completed at this time, they will be posted to the patient within 24 hours.

The Treatment, Support and Discharge Plan (TSDP) is developed with the patient and, if possible, PSP. A copy will be signed by the patient, PSP and staff member and then filed in the patient's healthcare record.

- The discharge checklist is to be completed by the allocated discharge nurse
- The Discharge National Outcomes Casemix Collection (NOCC) measures must be completed along with entering the Kessler10 in PSOLIS. If the patient has not completed the Kessler 10, record in PSOLIS as "Not Completed Due to Refusal by the Client").
- The Mental Health Care Transfer Summary must also be faxed/mailed to the relevant community mental health team, as indicated.
- A comprehensive discharge summary must be completed in the Notification and Clinical Summaries (NACS) on the day of discharge, and sent to the referrer, the GP and other treating clinicians as indicated. Copies of all discharge summaries written for a patient should be retained in the patient's healthcare record.
- A discharge iSoBAR handover is completed by the Clinical Nurse Consultant and emailed to birthnotificationscdis.cach@health.wa.gov.au to handover to the child health nurse.
- For WACHS patients, a discharge iSoBAR handover is completed by the Clinical Nurse Consultant and emailed to AreaOfficePopulationHealth.WACHS@health.wa.gov.au .

Procedure for Discharge to a Community Treatment Order

Refer to the [Clinician's Practice Guide to the Mental Health Act 2014](#).

Definitions

For the purpose of this guideline the following definitions apply.

- Discharge planning - includes the various activities instigated before the patient is discharged from the MBU.
- Personal support person (PSP) - a patient's partner, carer, close family member or other personal support person, guardian or enduring guardian, parent or legal guardian of a person under 18 years.

Related policies

[Carers Recognition Act 2004](#)

[Charter of Mental Health Care Principles](#)

[Chief Psychiatrist's Guidelines 2015](#)

[Chief Psychiatrist's Standards for Clinical Care 2015](#)

[Mental Health Act 2014](#)

[WNHS Discharge of Patient Policy](#)

Related WNHS procedures and guidelines

[Admission to MBU](#)

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NSQHS Standards Applicable:	 Std 1: Clinical Governance  Std 5: Comprehensive Care		 Std 6: Communicating for Safety  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Standards for Clinical Care		Transfer of Care			
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Version history

Version Notes	Date
First Published Version	October 2007
Regular review and updates	July 2015, June 2018
Clinical Practice Guideline: Discharge Planning rescinded	August 2021
Formatting and content changed to align with current WNHS procedure template. Document title changed to WNHS procedure: MBU Discharge Planning Procedure	August 2021
Three Yearly review - Amendments include: • minor formatting changes • updated hyperlinks and email addresses • discharge plan changed to Treatment Support and Discharge Plan (TSDP) • medical record change to healthcare record • added: patients own goals of admission • removed: National Standards for Mental Health Services and replaced with Chief Psychiatrist's Standards for Clinical Care to align with NMHS Mental Health documents.	May 2025

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